

Policy on Intensive Care and High Dependency Units

«Hospital Name»

Document control

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Prepared by	ICU/HDU In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Care in intensive care and high dependency units follows written guidance, uses defined admission and discharge criteria, and is backed by adequate staff, equipment, and infection control.

1. Purpose

This policy explains how «Hospital Name» provides ICU/HDU care based on written guidance, applies defined criteria for admitting, transferring, and discharging patients, keeps adequate trained staff and equipment available, handles bed shortages, follows infection prevention and control practices, and counsels patients and families.

This policy does not cover general infection control procedures or nursing staffing ratios in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all clinical and nursing staff working in the intensive care and high dependency units at «Hospital Name».

3. Policy standards

«Hospital Name» provides ICU/HDU care under written guidance that, at a minimum, covers everything in this policy, applies defined admission, transfer-in, discharge, and

transfer-out criteria, keeps adequate trained staff and necessary equipment available, has a procedure for bed shortages, follows infection prevention and control practices, runs a quality assurance programme, and counsels patients and families at least once a day.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not provide ICU/HDU care without written guidance based on standard treatment guidelines or sound clinical practice — at a minimum, this guidance must cover everything else in this policy.
- Do not admit, transfer in, discharge, or transfer out an ICU/HDU patient without applying the hospital's defined criteria for that decision.
- Do not leave a mechanism for "discharge against medical advice" undefined.
- Do not staff or equip the ICU/HDU with anything less than the necessary lifesaving and monitoring equipment and competent, trained staff, or without following applicable statutory requirements.
- Do not leave a bed-shortage situation without following the hospital's defined procedure for it.
- Do not depart from infection prevention and control practices consistent with standard treatment guidelines or sound clinical practice.
- Do not skip counselling the patient or family at least once a day, and whenever the patient's condition changes significantly, by a doctor of the treating team.
- Do not counsel a patient or family on a significant change without documenting it in writing or by audio-video recording.

5. What we do

5.1 Follow written guidance covering the whole standard

Care in the ICU/HDU is based on written guidance drawn from standard treatment guidelines or sound clinical practice for intensive/critical care. At a minimum, this guidance addresses everything else in this policy — admission and discharge criteria, staffing and equipment, bed-shortage procedure, infection control, quality assurance, and counselling. «Hospital Name» could adapt care-bundles where appropriate.

5.2 Apply defined admission, transfer, and discharge criteria

«Hospital Name» develops and adheres to criteria — based on physiologic and/or diagnostic parameters — covering four situations: admission, transfer-in, discharge, and transfer-out of ICU/HDU patients. There is also a defined mechanism for discharge against medical advice. Staff are trained to apply these criteria. National and international critical care society guidelines are a useful starting point.

5.3 Keep adequate staff and equipment

The ICU/HDU is equipped with all necessary lifesaving and monitoring equipment, handled by trained staff. Staff working in the ICU/HDU are competent, based on qualification and/or training. Applicable statutory requirements are followed. The exact equipment and staffing requirements are «Hospital Name»'s own determination, based on the scope and complexity of its services and guided by standard treatment guidelines or best clinical practice. The Indian Nursing Council's recommendations are a useful reference for nursing workforce planning.

5.4 Have a procedure for bed shortages

Whenever there are no vacant ICU beds and a patient needs one, «Hospital Name» follows a detailed, pre-established policy and procedure to address the situation — not something improvised at the time.

5.5 Follow infection prevention and control practices

Infection prevention and control practices in the ICU/HDU are consistent with standard treatment guidelines or sound clinical practice. Written guidance for this could stand alone or be part of the hospital's overall infection prevention and control manual. (Related requirements are covered in the hospital's other policies.)

5.6 Run a quality assurance programme

The quality assurance programme covers every aspect of ICU/HDU functioning, consistent with standard treatment guidelines or sound clinical practice. Care outcomes are monitored — for example, risk-adjusted standardised mortality rate, infection rates, re-admission rates, or re-intubation rates. These are examples; «Hospital Name» chooses the outcomes most relevant to its patients. National and international critical care society guidelines are a useful starting point for this programme, and the written guidance for it could stand alone or be part of the hospital's overall quality-improvement programme.

5.7 Counsel patients and families

A doctor from the treating team counsels the patient and/or family at least once a day, and whenever there's a significant change in the patient's condition. Counselling covers significant events since the last session, expected outcomes, and the family's queries about the patient's changing condition. Counselling is documented in writing, by audio-video recording, or both. It's preferable to also obtain an acknowledgement.

6. Stop-work authority

Do not admit, transfer, or continue managing an ICU/HDU patient when defined admission/discharge criteria are not applied, required minimum staffing or equipment is absent, or the bed-shortage escalation procedure has not been followed when no bed is available.

Stop-work applies to non-emergent admission/transfer decisions and elective bed allocation. A patient already in the ICU/HDU or in an active emergency continues receiving care while the gap is escalated.

The person who stops tells the ICU In-Charge and the Medical Superintendent the same shift. Refusing an unsafe ICU/HDU admission or continuation is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
ICU/HDU In-Charge	Oversees ICU/HDU operations; ensures written guidance, defined criteria for all four admission/transfer/discharge/transfer-out situations, bed-shortage procedure, IPC practices, and QA programme are in place; receives stop-work notifications the same shift.
Treating doctors (ICU/HDU)	Apply defined criteria for every admission, transfer-in, discharge, and transfer-out decision; counsel patients and families at least once a day and whenever the patient's condition changes significantly; document counselling in writing or by audio-video recording.
ICU/HDU nursing staff	Maintain care standards; follow IPC practices; assist with daily documentation; support quality assurance monitoring.

8. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance covering all required elements is in place and current
Criteria application	Defined criteria applied for all four situations (admission, transfer-in, discharge, transfer-out); DAMA mechanism defined and in use
Staffing and equipment	ICU/HDU staffed with competent personnel and equipped with required lifesaving and monitoring equipment; statutory requirements followed
Bed-shortage procedure	Pre-established procedure followed whenever ICU beds are unavailable; deviations escalated and documented

Infection prevention and control	IPC practices in ICU/HDU consistent with standard treatment guidelines or sound clinical practice
Quality assurance	Defined care outcomes monitored and analysed (e.g., mortality rates, infection rates, re-admission or re-intubation rates)
Counselling documentation	Daily counselling by treating team doctor documented; counselling on significant changes documented in writing or audio-video recording

9. Training and staff acknowledgement

All clinical and nursing staff working in the ICU/HDU shall be familiar with the written guidance applicable to their work, including admission and discharge criteria for all four situations, the bed-shortage procedure, infection prevention and control practices, the quality assurance programme, and daily counselling requirements.

I have read the Policy on Intensive Care and High Dependency Units of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical and nursing staff working in the intensive care and high dependency units.

11. Abbreviations

Abbreviation	Meaning
DAMA	Discharge Against Medical Advice
HDU	High Dependency Unit
ICU	Intensive Care Unit
IPC	Infection Prevention and Control
QA	Quality Assurance

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.9.a	Commitment*	Section 5.1 addresses

		written guidance for ICU/HDU care drawn from standard treatment guidelines or sound clinical practice, covering at a minimum all other elements of this standard.
COP.9.b	Commitment*	Section 5.2 addresses defined criteria covering four situations: admission, transfer-in, discharge, and transfer-out of ICU/HDU patients. Also addresses the defined mechanism for discharge against medical advice (DAMA) as a separate requirement. Stop-work authority (Section 6) also applies to this element.
COP.9.c	Commitment	Section 5.3 addresses adequacy of lifesaving and monitoring equipment, trained and competent ICU/HDU staff based on qualification and/or training, and compliance with applicable statutory requirements. Stop-work authority (Section 6) also applies to this element.
COP.9.d	Commitment*	Section 5.4 addresses the pre-established, detailed bed-shortage procedure followed whenever ICU beds are unavailable. Stop-work authority (Section 6) also applies to this element.
COP.9.e	Commitment*	Section 5.5 addresses infection prevention and control practices in the ICU/HDU consistent with standard treatment guidelines or sound clinical practice.
COP.9.f	Achievement*	Section 5.6 addresses the quality assurance programme covering all aspects of ICU/HDU functioning, with defined

		outcomes monitored (e.g., mortality rate, infection rates, re-admission and re-intubation rates).
COP.9.g	Commitment	Section 5.7 addresses counselling by a treating team doctor at least once a day and on every significant condition change, with documentation in writing or by audio-video recording.

13. Required Records/Evidence Checklist

Written guidance

- Written guidance for ICU/HDU care based on standard treatment guidelines or sound clinical practice, covering all required elements.

Admission, transfer, and discharge criteria

- Defined criteria (based on physiologic and/or diagnostic parameters) covering admission, transfer-in, discharge, and transfer-out.
- Defined mechanism for discharge against medical advice (DAMA).
- Evidence that staff are trained to apply the criteria.

Staffing and equipment

- Evidence of lifesaving and monitoring equipment in the ICU/HDU.
- Evidence that ICU/HDU staff are competent based on qualification and/or training.
- Evidence of compliance with applicable statutory requirements.

Bed-shortage procedure

- Documented bed-shortage procedure; records of application when beds are unavailable.

Infection prevention and control

- Written guidance for IPC practices in the ICU/HDU (standalone or part of the hospital's IPC manual).

Quality assurance programme

- Quality assurance programme documentation covering all aspects of ICU/HDU functioning.
- Records of defined care outcomes monitored and analysed.

Counselling records

- Daily counselling entries in patient records (in writing or audio-video recording).
- Counselling records for every significant change in patient condition.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.9.
2. National and international critical care society guidelines (reference for written guidance, criteria development, and quality assurance programme).
3. Indian Nursing Council recommendations (reference for nursing workforce planning in ICU/HDU).

Disclaimer

This policy reorganises the supplied COP.9 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The four-direction criteria (admission, transfer-in, discharge, transfer-out) and the separate DAMA mechanism are preserved exactly as stated. The exact phrase "at least once a day" for counselling frequency is preserved. No specific staffing ratios, equipment lists, or outcome thresholds have been invented; these are the hospital's own determination.